



TO SEND OUT

# Vytalix — One page for investors

Vytalix company-build documentation

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS\_BASE.

PREPARED FOR

The Founder, Vytalix

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VERSION

1.0 · Confidential draft

*For an angel or a fund deciding whether to take a first call. Send it as the attachment to a cold email.*

**Technology for Life** · UK health technology · pre-seed · [Founder name] · [email] · [phone] · [website]

## What Vytalix is

A British health-technology company with two layers. The cash layer is Advisory and Consulting: senior, founder-led help for health organisations, health-tech firms and charities, sold at £1,200–£1,800 a day or as fixed-price projects of £9,500–£28,000. The venture layer is MaternaLink, a maternity coordination product, in development.

Services pay for the company. MaternaLink is what an investor is buying.

## The problem

Maternity care fails in the gaps between people. UK inquiries keep finding the same three failures: nobody listened, nobody escalated, the handover was lost. Black women in the UK are more than twice as likely to die in pregnancy or childbirth as White women. Nigeria has more maternal deaths than any other country, mostly caused by delay rather than by a missing drug.

Hospital record systems do not close that gap. Consumer apps do not either. Donor messaging programmes reach millions of women at under a dollar each, but do nothing on the facility side.

## What we are building

MaternaLink version one: appointments and contact continuity, two-way messaging that escalates to a real person, clinician-approved education, and simple monitoring a midwife acts on. Works on a poor connection. Designed to sit beside the hospital record, not replace it.

It does not diagnose. That is deliberate, and it is what keeps version one out of medical-device regulation while evidence is built. A predictive AI risk score exists as a research project for later, behind data partnerships, ethics approval and a regulator's route.

## Where we are today, honestly

| Have                                                                         | Building                                                                               | Next                                                                            |
|------------------------------------------------------------------------------|----------------------------------------------------------------------------------------|---------------------------------------------------------------------------------|
| A full company plan: strategy, product spec, financial model, brand, website | Company registration, name clearance, and a written ownership deed for the product     | First paid advisory engagement, target day 45                                   |
| A working prototype dashboard, now under review                              | Advisory board: obstetrics, midwifery, NHS digital, Nigerian health system, regulation | One UK evaluation site and one Nigerian charity cohort in design, target day 90 |
| A researched list of 316 investors and a disciplined outreach process        | A published research report on maternity coordination in the UK and Nigeria            | SEIS and EIS advance assurance, applied for after registration                  |

What we do not have: revenue, customers, a signed pilot, a partner, an approval, or any clinical evidence. No hospital or government uses the prototype. We would rather you heard that from us.

## How the money works

Services are profitable from month two and need no product. The base scenario is £166,000 of revenue in year one, £488,000 in year two and £820,000 in year three, with services at 59% of the three-year total of £1.47m.

Product revenue starts at month 13 at the earliest, as paid pilots, then annual licences at £8–£40 per woman in the UK and £3–£12 in Africa. The careful scenario, services only, is £413,000 over three years and survives without a seed round.

## What we are raising, and what it buys

**£250,000–£500,000 pre-seed**, expected to qualify for SEIS and EIS (tax schemes that give British investors money back on an investment in a small company). It buys a product lead, the clinical safety and data protection work, the first build, and two pilot sites — about 18 months.

We will not open this round until three things are done: the company is registered, the product ownership deed is signed, and the first services revenue is in the bank. A seed round of £1.5m–£3m follows only on evidence from pilots.

## Why now

UK maternity safety and inequality are under national scrutiny, with money attached. Nigerian states are funding digital maternal programmes. Messaging is cheap enough that the constraint is coordination, not connectivity. And a services business means we can wait for the right pilot instead of taking the wrong one.

## The ask

Twenty minutes to test whether this fits your mandate. A data room is available once the company is registered and the ownership deed is signed. If the honesty above costs us the meeting, we would rather find out now.